Size: 165x120 mm

07/11/2023

ALLOR

Loratadine Oral Solution
5mg/5ml USP

Pharmacode

FORMULATION:
Each 5 ml contains:
Loratadine USP 5 mg

PHARMACOLOGY:
Loratadine, the active ingredient in Loratadine Oral Solution, is a tricyclic antihistamine with selective, peripheral H_1-receptor activity.
Loratadine has no clinically significant sedative or anticholinergic properties in the majority of the population and when used at the recommended dosage. During long-term treatment there were no clinically significant changes in vital signs, laboratory test values, physical examinations or electrocardiograms.
Loratadine has no significant H_2-receptor activity. It does not inhibit norepinephrine uptake and has practically no influence on cardiovascular function or on intrinsic cardiac pacemaker activity.

PHARMACOKINETICS:
After oral administration, loratadine is rapidly and well absorbed and undergoes an extensive first pass metabolism, mainly by CYP3A4 and CYP2D6. The major metabolite-desloratadine (DL)- is pharmacologically active and responsible for a large part of the clinical effect. Loratadine and DL achieve maximum plasma concentrations (T_{max}) between 1-1.5 hours and 1.5-3.7 hours after administration, respectively.
Increase in plasma concentrations of loratadine has been reported after concomitant use with ketoconazole, erythromycin, and cimetidine in controlled trials, but without clinically significant changes (including electrocardiographic).
Loratadine is highly bound (97% to 99%) and its active metabolite moderately bound (73% to 76%) to plasma proteins.
In healthy subjects, plasma distribution half-lives of loratadine and its active metabolite are approximately 1 and 2 hours, respectively. The mean elimination half lives in healthy adult subjects were 8.4 hours (range=3 to 20 hours) for loratadine and 28 hours (range-8.8 to 92 hours for the major active metabolite).
Approximately 40% of the dose is excreted in the urine and 42% in the faeces over a 10 day period and mainly in the form of conjugated metabolites. Approximately 27% of the dose is eliminated in the urine during the first 24 hours. Less than 1% of the active substance is excreted unchanged in active form, as loratadine or DL.
The bioavailability parameters of loratadine and of the active metabolite are dose proportional.
The pharmacokinetic profile of loratadine and its metabolites is comparable in healthy adult volunteers and in healthy geriatric volunteers.
Concomitant ingestion of food can delay slightly the absorption of loratadine but without influencing the clinical effect.
In patients with chronic renal impairment, both the AUC and peak plasma levels (C_{max}) increased for loratadine and its metabolite as compared to the AUCs and peak plasma levels (C_{max}) of patients with normal renal function. The mean elimination half-lives of loratadine and its metabolite were not significantly different from that observed in normal subjects. Haemodialysis does not have an effect on the pharmacokinetics of loratadine or its active metabolite in subjects with chronic renal impairment.
In patients with chronic alcoholic liver disease, the AUC and peak plasma levels (C_{max}) of loratadine were double while the pharmacokinetic profile of the active metabolite was not significantly changed from that in patients with normal liver function. The elimination half-lives for loratadine and its metabolite were 24 hours and 37 hours, respectively, and increased with increasing severity of liver disease.
Loratadine and its active metabolite are excreted in the breast milk of lactating women.

INDICATION:
Loratadine Oral Solution is indicated for the symptomatic treatment of allergic rhinitis and chronic idiopathic urticarial in adults and children over the age of 2 years.

DOSAGE AND ADMINISTRATION:
Adults and children over 12 years of age:
10ml (10mg) of the syrup once daily.
Paediatric population
Children 2 to 12 years of age are dosed by weight:
Body weight more than 30kg: 10ml (10mg) of the syrup once daily;
Body weight 30kg or less: 5ml (5mg) of the syrup once daily.
Efficacy and safety of Loratadine Oral Solution in children under 2 years of age has not been established.
Patients with severe liver impairment
Patients with severe liver impairment should be administered a lower initial dose because they may have reduced clearance of loratadine. An initial dose of 10mg every other day is recommended for adults and children weighing more than 30kg, and for children weighing 30kg or less, 5ml (5mg) every other day is recommended.
Patients with severe renal impairment
No dosage adjustments are required in the elderly or in patients with renal insufficiency.
Elderly
No dosage adjustments are required in the elderly.
Method of administration
For oral administration.

CONTRAINDICATION:
Loratadine Oral Solution is contraindicated in patients who are hypersensitive to the active substance or to any of the excipients in this formulation

PRECAUTIONS:
Loratadine Oral Solution should be administered with caution in patients with severe liver impairment.

This medicinal product contains sucrose; patients with rare hereditary problems of fructose intolerance, glucose-galactose malabsorption or sucrase-isomaltase insufficiency should not take this medicine

PREGNANCY:
Pregnancy
A large amount of data on pregnant women (more than 1000 exposed outcomes) indicate no malformative nor feto/neonatal toxicity of loratadine. Animal studies do not indicate direct or indirect harmful effects with respect to reproductive toxicity. As a precautionary measure, it is preferable to avoid the use of Loratadine Oral Solution during pregnancy.
Breast-feeding
Loratadine is excreted in breast milk, therefore the use of loratadine is not recommended in breast-feeding women.
Fertility
There are no data available on male and female fertility.

ADVERSE EFFECTS:
a. Summary of the safety profile
In clinical trials involving adults and adolescents in a range of indications including AR and CIU, at the recommended dose of 10mg daily, adverse reactions with loratadine were reported in 2% of patients in excess of those treated with the placebo. The most frequent adverse reactions reported in excess of placebo were somnolence (1.2%), headache (0.6%), increased appetite (0.5%) and insomnia (0.1%).
b. Tabulated list of adverse reactions
The following adverse reactions reported during the post-marketing period are listed in the following table by System Organ Class. Frequencies are defined as very common (≥ 1/10), common (≥ 1/100 to < 1/10), uncommon (≥ 1/1,000 to < 1/100), rare (≥ 1/10,000 to < 1/1,000), very rare (< 1/10,000) and not known (cannot be estimated from the available data).
Within each frequency grouping, adverse reactions are presented in order of decreasing seriousness.

System Organ Class	Frequency	Adverse Reaction
Immune system disorders	Very rare	Hypersensitivity reactions (including angioedema and anaphylaxis)
Nervous system disorders	Very rare	Dizziness, Convulsion
Cardiac disorders	Very rare	Tachycardia, palpitation
Gastrointestinal disorders	Very rare	Nausea, dry mouth, gastritis
Hepato-biliary disorders	Very rare	Abnormal hepatic function
Skin and subcutaneous tissue disorders	Very rare	Rash, alopecia
General disorders and administration site conditions	Very rare	Fatigue

Pharmacode

c. Paediatric population
In clinical trials in a paediatric population children aged 2 through 12 years, com adverse reactions reported in excess of placebo were headache (2.7%), nervo (2.3%), and fatigue (1%).

DRUG INTERACTION:
When administered concomitantly with alcohol, Loratadine Oral Solution has no potentiating effects as measured by psychomotor performance studies.
Potential interaction may occur with all known inhibitors of CYP3A4 or CYP2D6 resulting in elevated levels of loratadine, which may cause an increase in adverse events.
Increase in plasma concentrations of loratadine has been reported after concomitant use with ketoconazole, erythromycin, and cimetidine in controlled trials, but without clinically significant changes (including electrocardiographic).
Paediatric population
Interaction studies have only been performed in adults.

STORAGE CONDITON:
Store at temperatures not exceeding 30°C.
Jauhkan daripada capaian kanak-kanak.
Keep out of reach of children.

XL
Manufactured by :
XL LABORATORIES PVT. LTD.
E-1223, Phase-I Extn., (Ghatal) RIICO Indl. Area,
Bhiwadi-301019, Rajasthan, India

Product Registration Holder / Importer:
Unimed Sdn Bhd,
No.53, Jalan Tembaga SD 5/2B,
Bandar Sri Damansara,
52200 Kuala Lumpur.

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